



Two Roads

Staying in your own home, or moving into a continuing care home. What each one gives you, what each one costs, and how to start either.

Most guides about continuing care are written for families moving an elderly parent into a facility. This one is not. It is written for people deciding between two roads, including the many who want to stay exactly where they are and need help to do it.

Both roads are real. Both are publicly funded. And both of them start at the same phone call.

One door, two roads

Call Health Link at 811 and ask for a continuing care needs assessment.

No referral. No doctor's letter. No form to fill in first. No cost.

A nurse or case manager assesses what you need. That single assessment decides what you can get *at home* and what you could get *in a continuing care home*. It is the door to both roads, not just one.

Have ready: your Alberta Health Care number, your address, your contact details. That is the whole list.

Age has nothing to do with it. Alberta's own page says any Albertan can receive continuing care services, whatever their age, whatever their diagnosis, however long they need it. Eligibility rests on assessed unmet care needs. A great many disabled adults never ring because they assume this is a seniors' system. It is not.

You do not have to know which road you want before you ring. Working that out is what the assessment is for.

The money, told straight

This is the part nobody lays out side by side, and it is usually the thing that decides.

If you stay in your own home

The care hours you are assessed as needing are publicly funded and **there is no client fee for them**. Alberta does not charge an income-tested fee for assessed home care, which is unusual in Canada and worth knowing.

You keep your income. You also keep every cost of running your own life: rent or mortgage, utilities, food, and anything above your assessed hours.

If you move into a continuing care home

Your care is publicly funded. What you pay is the **accommodation charge** — the room, meals, laundry, housekeeping, utilities and building costs. The Alberta government sets the maximum, and it is the same at every home in the province, public or private.

What the accommodation charge is, and the number underneath it

As of 1 April 2026, Alberta listed maximum daily charges of **\$70.60 for a shared room**, \$81.60 private, \$97.70 for a one-bedroom suite and \$112.45 for a multi-bedroom suite.

Those figures went up again on 1 August 2026 by an inflation adjustment of 1.8 per cent. Read the current table off alberta.ca/continuing-care-accommodation-charges before relying on any number here. The charge rises every year.

A shared room at the April rate works out at roughly **\$2,148 a month**.

Now put that beside the benefit. AISH is \$1,940 a month. ADAP is \$1,740. **The cheapest accommodation charge in continuing care is more than the entire monthly disability benefit.**

So a subsidy has to carry the difference, and Alberta states the floor plainly. As of January 2026, residents eligible for these benefits are **assured of at least \$373 per month in disposable income** after the accommodation charge is paid.

\$373 a month. That is what is left for clothes, a phone, transportation, a haircut, a gift for a grandchild, and anything the home does not provide. Not a penalty. The design.

Ask for the financial screening rather than waiting to be offered it. The subsidy is not automatic if nobody assesses you for it.

The two roads, side by side

Everything below comes from Alberta's own published standards and pages. Where the standards protect one road and not the other, that is said plainly.

	Staying in your own home	A continuing care home
How you start	Call 811. Continuing care needs assessment.	Call 811. The same assessment.
Who decides	A nurse or case manager, using a standardised assessment tool.	The same assessment, then placement is coordinated through the health authority.
What care costs you	Nothing for your assessed hours. No client fee.	Nothing for care. Care is publicly funded.
What you pay for	Your own rent, utilities, food, and any hours above what you were assessed for.	The accommodation charge. Room, meals, laundry, housekeeping, utilities.
What you keep	Your income, less your own living costs.	A floor of \$373 a month in disposable income.
Reassessment	Yearly, and whenever your health significantly changes.	Type B yearly. Type A every three months. Plus on any significant change.
A named case manager	Required. You must be told who to contact.	Required. You must be told who to contact.
Your care plan	Yours to see on request, and to help write.	Yours to see on request, and to help write.
Equipment	Must be provided, or you must be referred to someone who can provide it.	Must be provided, or you must be referred.
Bathing	No published frequency standard.	At least twice a week, by the method you prefer.
Oral care	No published frequency standard.	Offered twice a day, more if your care plan says so.
Activities	Not covered by the standards.	Life enrichment services, as set out in your care plan.
Doctor on call	Not covered by the standards.	Type A homes must have a documented on-call procedure.
If services are disrupted	The operator must have an emergency and contingency plan.	Same, plus a named responsible person who must be on site.
Living at risk	Your choice must be recorded in a signed managed risk agreement.	Same. The right applies on both roads.

Read that table again, because there is a finding in it

Look at the four rows with no standard on the left. Bathing frequency. Oral care. Activities. A doctor on call.

Alberta's health service standards protect you more inside a facility than they do in your own home. At home you are entitled to an assessment, a case manager, a care plan, equipment, and the right to live at risk. You are not entitled to a bath twice a week.

That is not an argument for going into a facility. It is an argument for knowing what your assessed hours cover before you agree to them, and for getting the specifics written into your care plan, because the care plan is where your entitlement lives when the standards are silent.

Road one: staying in your own home

What it is called now

Alberta calls it **home and community care**. You may still hear the old name, the Coordinated Home Care Program, or simply home care. Type 1 is delivered directly by the health authority. Type 2 is delivered by a provider under agreement with it.

What you can be assessed for

- Nursing care and personal care.
- Rehabilitation and therapy.
- Case management, coordinating the rest.
- Assistive equipment you are assessed as needing, or a referral to someone who provides it.

Two ways to receive it

Client-Directed Home Care Invoicing, and Self-Managed Care

Client-Directed Home Care Invoicing lets you choose an approved provider who bills Alberta Blue Cross directly for your funded hours. Available in Central and Northern Alberta. This is the simpler of the two and it is the one most people want.

Self-Managed Care gives you the funds and you hire and manage your own workers. You become the employer, with the payroll, scheduling and record-keeping that comes with it. Capped at 263 hours a month or \$4,500.

Both start from the same 811 assessment. Ask about them by name, because you may not be offered them.

The trade-off worth understanding: self-managed care gives you the most control over who comes into your home and when. It also makes you an employer. For some people that control is worth the paperwork. For others it is one more unpaid job. Neither answer is wrong.

Making the home itself work

Assessed hours do not fix a bathroom you cannot use. The Residential Access Modification Program pays for permanent accessibility work — ramps, stair lifts, wider doorways, bathroom changes.

- **Up to \$12,000 per benefit year**, and up to \$24,000 over ten years, as of 21 August 2026.
- **Renters qualify**, with a landlord-signed property modification agreement. You do not have to own your home.
- It can fund modifications to **a caregiver's home**, not only your own.
- You need a rehabilitation professional, such as an occupational therapist, to complete a recommendation form. Decision within 30 days.
- Eligibility keys on wheelchair use, or being 65 and over using a four-wheel walker, or a listed progressive neurological condition. If yours is not on the list, ask your specialist to document how you meet the criteria rather than assuming you are out.

The limit of this road, said plainly

Assessed hours are rationed. They cover specific tasks a set number of times a week, not continuous presence. If what you need is someone there at unpredictable times, home care as assessed may not reach it, and the gap gets filled by family, by paying privately, or not at all.

Knowing that before you build a plan around it is better than finding out in month three.

Road two: a continuing care home

The three types, and the old words for them

The law changed on 1 April 2024. Alberta Health Services warns that its own pages still carry the old language, so you will hear both.

Type	What it used to be called	Who it is for
Type B	Formerly designated supportive living, or DSL.	For people with various levels of medical need who cannot stay safely at home or in a supportive living accommodation. Care aides and licensed practical nurses on site around the clock.
Type B Secure	Formerly DSL4-Dementia.	Type B with a secured area, for people who need specialised dementia care.
Type A	Formerly long-term care, nursing home, auxiliary hospital.	For complex medical needs. A registered nurse on site 24 hours a day.
Type C	Newly brought under the same legislation.	Publicly funded hospice, for specialised end-of-life care.

Do not rule yourself out on the word stable

Some private placement companies describe Type B as being for people who are *medically and physically stable*.

Alberta's own wording is wider. Type B continuing care homes are for people with *various levels of medical needs* who cannot remain safely in supportive living or at home.

If a company's checklist tells you that you would not qualify, that is not the test. **The assessment is the test**, and the assessment is free.

Supportive living is a different thing again

Supportive living accommodation without a Type A or B designation is not the same as a continuing care home. It is a congregate setting — seniors lodges, some group homes — where residents live as independently as they can and arrange publicly funded home care separately. Cost is set by the operator, not by the province.

This is the category people most often confuse with the others, and the one where a private-pay bill can arrive unexpectedly. Ask which of the three you are being shown, and get the answer in writing.

What you are owed on either road

These are not courtesies. They sit in the Continuing Care Health Service Standards, made under the Continuing Care Act, and they bind every operator receiving public funding, in your home or in a facility.

Eight things you are entitled to, and most people are never told

A named case manager. Every client must have an assigned regulated health professional responsible for coordinating your care, and you must be told who to contact with questions.

Your care plan, on request. You may see it, help write it, and invite anyone you choose to help — a friend, an advocate, a family member.

Written information about what is funded and what is not, including any cost assigned to you, when your care starts. Not verbally. In writing.

Written information about outside complaint routes — not only the operator's internal process — when your care starts and any time you ask.

Equipment you are assessed as needing, or a referral to a service that can provide it.

A reassessment when your health significantly changes, not only on the annual schedule.

An interdisciplinary team meeting to review your care plan yearly and whenever your health significantly changes.

The right to live at risk. See below. This is the most important one in the document.

The right to live at risk

If you understand a risk and accept it, the standards do not let an operator simply overrule you. What they require instead is a **managed risk agreement**: a signed, dated document recording the risk, what will be done to manage it, and your decision.

The standards define living at risk as the client understanding the facts of a situation, the risks of their decision, and accepting the possible outcomes. You are entitled to a signed copy, and it must be reviewed at your team conference.

This matters because it is the written answer to the sentence disabled people hear most often in this system: *you cannot stay where you are*. Sometimes that is true. But it is a conversation with a process attached, not a verdict, and the process is supposed to put your decision on paper rather than around it.

How to start, on either road

The same four steps whichever road you end up on.

- **Ring 811** and ask for a continuing care needs assessment. Have your Alberta Health Care number ready. Say plainly what you cannot manage, and say it in terms of tasks and days, not diagnoses. What can you not do, how often does it go wrong.
- **Write it down before you ring.** Falls in the last six months. Medication mistakes. Which parts of washing, dressing, toileting, getting out of bed, cooking and eating are a problem, and how often. If someone is helping you now, say what would happen if they stopped.
- **Ask three questions at the end of the call.** What is the wait for the in-person assessment. Who is my case manager and how do I reach them. Can home care start or increase while I wait.
- **Write down who you spoke to and when.** Every time. That record is what you rely on in three months when nobody remembers the conversation.

If you are told no, or told nothing

In order. Each step creates the record the next one needs.

- **Ask for the decision in writing**, with the reason. This single step changes more outcomes than any other.
- **The operator's concerns process**. They must give you their policy, and must also give you written information about outside complaint routes. Ask for both.
- **AHS Patient Relations**, for concerns about care or an unsuitable option you were offered. They respond within three business days.
- **The Office of the Alberta Health Advocates**, which can review conduct against the Alberta Health Charter.
- **Appeals**. A continuing care decision can be appealed and the window is short. Confirm the deadline on your decision letter the day it arrives. Appeals Secretariat 780-427-2709, or 310-0000 toll-free.
- **Alberta Ombudsman**, for fairness once other routes are used up. 403-297-6185.
- **Your MLA's constituency office**. Staff help with stuck provincial files, whoever you vote for.

What we could not confirm, said plainly

Wait times are not published for assessment or for placement. Anyone who gives you a number is estimating. Ask, and write down who told you what and when.

The zone intake numbers are not in this guide. Numbers circulating online for zone continuing care access come from service-finder pages rather than a published directory, and a wrong intake number sends a person in crisis to a dead line. **811 is published everywhere and works**. Use it.

The August 2026 accommodation charges are not printed here. The increase is confirmed at 1.8 per cent from 1 August 2026, but the current table should be read off alberta.ca rather than calculated.

Whether the \$373 floor rises with the accommodation charge is not established. The charge is adjusted annually. Nothing located says the floor is.

Figures in this guide are what each body listed on its own site as of 13 September 2026. If something has changed since, the body's own page is where to check it.

*Wanting to stay in your own home is not stubbornness, and needing more help than home can give is not failure.
Both roads are real. You are allowed to ask what is on each one before you choose.*

The Alberta Disability System Breakdown
albertadisabilitybreakdown@outlook.com

Compiled September 2026 from the Continuing Care Act and its Health Service Standards, Alberta.ca continuing care and accommodation charge pages, and Alberta Health Services. Verify anything time-sensitive before relying on it.